BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. BRAF tumour status is checked for the advanced or adjuvant melanoma indication.	Blood count, kidney/liver tests and glucose before every 4-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue immune-toxicity monitoring for at least five months after the last dose, and longer when advised. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00484 v13a: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Nivolumab · every 4 weeks · adjuvant melanoma: treatment and checks

Completely resected melanoma with lymph-node involvement or metastatic disease; the team confirms the stage III or resected stage IV indication. Uveal melanoma is excluded from this adjuvant pathway.

Nivolumab every 4 weeks. Adjuvant treatment continues for up to 12 months, subject to safety review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP	Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. BRAF tumour status is checked for the advanced or adjuvant melanoma indication.	Blood count, kidney/liver tests and glucose before every 4-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue immune-toxicity monitoring for at least five months after the last dose, and longer when advised. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan.
Cancer, skin and daily function NCCP	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.

What each check is for

1 Adjuvant treatment after surgery

Every 4 weeks

Nivolumab helps immune cells act against melanoma. After complete surgery, its aim is to lower the risk of recurrence; this does not guarantee that melanoma will not return.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.41